

Name : Mr. HIMANSHU  
Lab No. : 494249770  
Ref By : Self  
Collected : 2/3/2026 7:25:00AM  
A/c Status : P  
Collected at : VASUNDHARA LAB HOME VISIT  
GROUND FLOOR, PLOT NO.20 NEAR SBI BANK,  
VASUNDHRA, GHAZIABAD  
Contact No-01203988505

Age : 43 Years  
Gender : Male  
Reported : 2/3/2026 11:42:21AM  
Report Status : Final  
Processed at : LPL-VASUNDHARA LAB  
GF- Commercial Premises Viz 15C  
/20,Vasundhara , Ghaziabad. U.P 201012



ID : A2602260278

### Test Report

Test Name Results Units Bio. Ref. Interval

#### PACKAGE NAME- T6

#### HEMOGRAM

|                                                             |              |          |                |
|-------------------------------------------------------------|--------------|----------|----------------|
| Hemoglobin<br>(Photometry)                                  | 14.18        | g/dL     | 13.00 - 17.00  |
| Packed Cell Volume (PCV)<br>(Calculated)                    | 42.00        | %        | 40.00 - 50.00  |
| RBC Count<br>(Electrical impedance)                         | 4.76         | mill/mm3 | 4.50 - 5.50    |
| MCV<br>(Electrical impedance)                               | 88.20        | fL       | 83.00 - 101.00 |
| Mentzer Index<br>(Calculated)                               | 18.5         |          |                |
| MCH<br>(Calculated)                                         | 29.80        | pg       | 27.00 - 32.00  |
| MCHC<br>(Calculated)                                        | 33.80        | g/dL     | 31.50 - 34.50  |
| Red Cell Distribution Width (RDW)<br>(Electrical Impedance) | 13.90        | %        | 11.60 - 14.00  |
| Total Leukocyte Count (TLC)<br>(Electrical Impedance)       | 7.28         | thou/mm3 | 4.00 - 10.00   |
| <b>Differential Leucocyte Count (DLC)</b>                   |              |          |                |
| Segmented Neutrophils<br>(Optical/Impedance)                | 48.66        | %        | 40.00 - 80.00  |
| Lymphocytes<br>(Optical/Impedance)                          | <b>40.57</b> | %        | 20.00 - 40.00  |
| Monocytes<br>(Optical/Impedance)                            | 8.28         | %        | 2.00 - 10.00   |
| Eosinophils<br>(Optical/Impedance)                          | 2.19         | %        | 1.00 - 6.00    |
| Basophils<br>(Optical/Impedance)                            | 0.30         | %        | <2.00          |
| <b>Absolute Leucocyte Count</b>                             |              |          |                |
| Neutrophils<br>(Calculated)                                 | 3.54         | thou/mm3 | 2.00 - 7.00    |
| Lymphocytes<br>(Calculated)                                 | 2.95         | thou/mm3 | 1.00 - 3.00    |
| Monocytes<br>(Calculated)                                   | 0.60         | thou/mm3 | 0.20 - 1.00    |

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|                                                 |                                          |
|-------------------------------------------------|------------------------------------------|
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| VASUNDHRA, GHAZIABAD                            |                                          |
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| Eosinophils<br>(Calculated)                 | 0.16    | thou/mm3 | 0.02 - 0.50        |
| Basophils<br>(Calculated)                   | 0.02    | thou/mm3 | 0.02 - 0.10        |
| Platelet Count<br>(Electrical impedance)    | 208     | thou/mm3 | 150.00 - 410.00    |
| Mean Platelet Volume<br>(Coulter Principle) | 10.2    | fL       | 6.5 - 12.0         |
| E.S.R.                                      | 3       | mm/hr    | 0 - 15             |

### Comment

In anaemic conditions Mentzer index is used to differentiate Iron Deficiency Anaemia from Beta- Thalassemia trait. If Mentzer Index value is >13, there is probability of Iron Deficiency Anaemia. A value <13 indicates likelihood of Beta- Thalassemia trait and Hb HPLC is advised to rule out the Thalassemia trait.

### Note

- As per the recommendation of International council for Standardization in Hematology, the differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood
- Test conducted on EDTA whole blood

Predominantly Normocytic normochromic RBCs

WBC is normal in number.

Platelets are adequate.



|                                                 |                                          |
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### Test Report

| Test Name                                               | Results | Units | Bio. Ref. Interval |
|---------------------------------------------------------|---------|-------|--------------------|
| <b>LIVER &amp; KIDNEY FUNCTION TEST (LFT &amp; KFT)</b> |         |       |                    |
| BUN/Creatinine Ratio<br>(Calculated)                    | 10      |       |                    |

### Note

1. Estimated GFR (eGFR) calculated using the 2021 CKD-EPI creatinine equation and GFR Category reported as per KDIGO guideline 2012.
2. eGFR category G1 or G2 does not fulfil the criteria for CKD, in the absence of evidence of kidney damage
3. The BUN-to-creatinine ratio is used to differentiate prerenal and postrenal azotemia from renal azotemia. Because of considerable variability, it should be used only as a rough guide. Normally, the BUN/creatinine ratio is about 10:1
4. Test conducted in Serum



|                       |                                                |                        |                                               |
|-----------------------|------------------------------------------------|------------------------|-----------------------------------------------|
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| <b>Lab No.</b> :      | <b>494249770</b>                               | <b>Gender</b> :        | <b>Male</b>                                   |
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|                       | <b>GROUND FLOOR, PLOT NO.20 NEAR SBI BANK,</b> |                        | <b>/20,Vasundhara , Ghaziabad. U.P 201012</b> |
|                       | <b>VASUNDHRA, GHAZIABAD</b>                    |                        |                                               |
|                       | <b>Contact No-01203988505</b>                  |                        |                                               |
| <b>ID</b> :           | <b>A2602260278</b>                             |                        |                                               |

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|---------------------------------------------------------|--------------|---------------|--------------------|
| <b>LIVER &amp; KIDNEY FUNCTION TEST (LFT &amp; KFT)</b> |              |               |                    |
| Creatinine<br>(Jaffe Compensated)                       | 0.83         | mg/dL         | <1.20              |
| GFR Estimated<br>(CKD EPI Equation 2021)                | 111          | mL/min/1.73m2 | >59                |
| GFR Category<br>(KDIGO Guideline 2012)                  | G1           |               |                    |
| Urea<br>(Urease UV)                                     | <b>18.00</b> | mg/dL         | 19.00 - 44.00      |
| Urea Nitrogen Blood<br>(Urease UV)                      | <b>8.41</b>  | mg/dL         | 8.90 - 20.60       |
| Uric Acid<br>(Enzymatic Colorimetric)                   | <b>7.21</b>  | mg/dL         | 3.4 - 7.0          |
| AST (SGOT)<br>(IFCC without p5p)                        | 29.0         | U/L           | <40                |
| ALT (SGPT)<br>(IFCC without p5p)                        | 30.4         | U/L           | <41                |
| AST:ALT Ratio<br>(Calculated)                           | 0.95         |               | <1.00              |
| GGTP<br>(IFCC)                                          | 30.3         | U/L           | <71.00             |
| Alkaline Phosphatase (ALP)<br>(IFCC)                    | 69.30        | U/L           | <128               |
| Bilirubin Total<br>(Diazo)                              | 0.67         | mg/dL         | <1.10              |
| Bilirubin Direct<br>(Diazo)                             | <b>0.29</b>  | mg/dL         | <0.20              |
| Bilirubin Indirect<br>(Calculated)                      | 0.38         | mg/dL         | <1.10              |
| Total Protein<br>(Biuret)                               | 7.19         | g/dL          | 6.40 - 8.30        |
| Albumin<br>(Biuret)                                     | 4.42         | g/dL          | 3.50 - 5.20        |
| Globulin(Calculated)                                    | 2.77         | gm/dL         | 2.0 - 3.5          |
| A : G Ratio<br>(Calculated)                             | 1.60         |               | 0.90 - 2.00        |
| Calcium, Total<br>(NM-BAPTA)                            | 9.34         | mg/dL         | 8.6 - 10.0         |





|                                                 |                                          |
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|------------------------------|---------|-------|--------------------|
| Phosphorus<br>(Molybdate UV) | 3.99    | mg/dL | 2.6 - 4.5          |
| Sodium<br>(ISE)              | 140.90  | mEq/L | 136.00 - 145.00    |
| Potassium<br>(ISE)           | 4.05    | mEq/L | 3.5 - 5.1          |
| Chloride<br>(ISE, Indirect)  | 103.00  | mEq/L | 98 - 108           |

### Note

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### Test Report

| Test Name                              | Results | Units | Bio. Ref. Interval |
|----------------------------------------|---------|-------|--------------------|
| <b>HbA1c</b><br>(HPLC, NGSP certified) |         |       |                    |
| HbA1c                                  | 5.7     | %     | 4.00 - 5.60        |
| Estimated average glucose (eAG)        | 117     | mg/dL |                    |

### Interpretation

HbA1c result is suggestive of at risk for Diabetes (Prediabetes)/ well controlled Diabetes in a known Diabetic

### Interpretation as per American Diabetes Association (ADA) Guidelines

| Reference Group | Non diabetic adults $\geq 18$ years | At risk (Prediabetes) | Diagnosing Diabetes | Therapeutic goals for glycemic control |
|-----------------|-------------------------------------|-----------------------|---------------------|----------------------------------------|
| HbA1c in %      | 4.0-5.6                             | 5.7-6.4               | $\geq 6.5$          | $< 7.0$                                |

### Note

1. Presence of Hemoglobin variants and/or conditions that affect red cell turnover must be considered, particularly when the HbA1C result does not correlate with the patient's blood glucose levels.
2. Test conducted in EDTA Whole blood

| FACTORS THAT INTERFERE WITH HbA1C MEASUREMENT                                                                                                                                                              | FACTORS THAT AFFECT INTERPRETATION OF HbA1C RESULTS                                                                                                                                                                                                                                                 |
|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Hemoglobin variants, elevated fetal hemoglobin (HbF) and chemically modified derivatives of hemoglobin (e.g. carbamylated Hb in patients with renal failure) can affect the accuracy of HbA1c measurements | Any condition that shortens erythrocyte survival or decreases mean erythrocyte age (e.g., recovery from acute blood loss, hemolytic anemia, HbSS, HbCC, and HbSC) will falsely lower HbA1c test results regardless of the assay method used. Iron deficiency anemia is associated with higher HbA1c |



|                                                 |                                               |
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### Test Report

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|------------------------------------------------|---------|-------|--------------------|
| <b>VITAMIN B12 (CYANOCOBALAMIN)</b><br>(ECLIA) | 303.00  | pg/mL | 211.00 - 946.00    |

#### Notes

1. Interpretation of the result should be considered in relation to clinical circumstances.
2. It is recommended to consider supplementary testing with plasma Methylmalonic acid (MMA) or plasma homocysteine levels to determine biochemical cobalamin deficiency in presence of clinical suspicion of deficiency but indeterminate levels. Homocysteine levels are more sensitive but MMA is more specific
3. False increase in Vitamin B12 levels may be observed in patients with intrinsic factor blocking antibodies, MMA measurement should be considered in such patients
4. The concentration of Vitamin B12 obtained with different assay methods cannot be used interchangeably due to differences in assay methods and reagent specificity
5. Test conducted on serum

|                                          |              |        |                |
|------------------------------------------|--------------|--------|----------------|
| <b>VITAMIN D 25 - HYDROXY</b><br>(ECLIA) | <b>26.90</b> | nmol/L | 75.00 - 250.00 |
|------------------------------------------|--------------|--------|----------------|

#### Interpretation

| LEVEL                  | REFERENCE RANGE<br>IN nmol/L | COMMENTS                                                                   |
|------------------------|------------------------------|----------------------------------------------------------------------------|
| Deficient              | < 50                         | High risk for developing bone disease                                      |
| Insufficient           | 50-74                        | Vitamin D concentration which normalizes Parathyroid hormone concentration |
| Sufficient             | 75-250                       | Optimal concentration for maximal health benefit                           |
| Potential intoxication | >250                         | High risk for toxic effects                                                |

#### Note

- The assay measures both D2 (Ergocalciferol) and D3 (Cholecalciferol) metabolites of vitamin D.



|                                                 |                                               |
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|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------|-------|--------------------|
| <ul style="list-style-type: none"> <li>25 (OH)D is influenced by sunlight, latitude, skin pigmentation, sunscreen use and hepatic function.</li> <li>Optimal calcium absorption requires vitamin D 25 (OH) levels exceeding 75 nmol/L.</li> <li>It shows seasonal variation, with values being 40-50% lower in winter than in summer.</li> <li>Levels vary with age and are increased in pregnancy.</li> <li>A new test Vitamin D, Ultrasensitive by LC-MS/MS is also available</li> <li>Test conducted on serum</li> </ul> |         |       |                    |

### Comments

Vitamin D promotes absorption of calcium and phosphorus and mineralization of bones and teeth. Deficiency in children causes Rickets and in adults leads to Osteomalacia. It can also lead to Hypocalcemia and Tetany. Vitamin D status is best determined by measurement of 25 hydroxy vitamin D, as it is the major circulating form and has longer half life (2-3 weeks) than 1,25 Dihydroxy vitamin D (5-8 hrs).

### Decreased Levels

- Inadequate exposure to sunlight
- Dietary deficiency
- Vitamin D malabsorption
- Severe Hepatocellular disease
- Drugs like Anticonvulsants
- Nephrotic syndrome

### Increased levels

Vitamin D intoxication





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| GLUCOSE FASTING (F)<br>(Hexokinase) | 93.70   | mg/dL | 70.00 - 100.00     |

**Note:**Test conducted in Plasma



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|--------------------------------------|-------------|--------|--------------------|
| <b>THYROID PROFILE TOTAL (ECLIA)</b> |             |        |                    |
| T3, Total                            | 1.26        | ng/mL  | 0.80 - 2.00        |
| T4, Total                            | 8.55        | µg/dL  | 5.10 - 14.10       |
| TSH                                  | <b>4.49</b> | µIU/mL | 0.27 - 4.20        |

### Note:

1. TSH levels are subject to circadian variation, reaching peak levels between 2 - 4.a.m. and at a minimum between 6-10 pm . The variation is of the order of 50% . hence time of the day has influence on the measured serum TSH concentrations.
2. Alteration in concentration of Thyroid hormone binding protein can profoundly affect Total T3 and/or Total T4 levels especially in pregnancy and in patients on steroid therapy.
3. Unbound fraction ( Free,T4 /Free,T3) of thyroid hormone is biologically active form and correlate more closely with clinical status of the patient than total T4/T3 concentration
4. Values <0.03 uIU/mL need to be clinically correlated due to presence of a rare TSH variant in some individuals
5. Test conducted on serum



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| <b>IRON STUDIES</b><br>(Ferrozine, calculated) |         |       |                    |
| Iron                                           | 109.00  | µg/dL | 59.00 - 158.00     |
| Total Iron Binding Capacity (TIBC)             | 356.00  | µg/dL | 228.00 - 428.00    |
| Transferrin Saturation                         | 30.62   | %     | 20.00 - 50.00      |
| UIBC                                           | 247.00  | µg/dL | 112.00 - 346.00    |

**Note:** Test conducted on serum

#### Comments

**Iron** is an essential trace mineral element which forms an important component of hemoglobin, metallocompounds and Vitamin A. Deficiency of iron, leads to microcytic hypochromic anemia. The toxic effects of iron are deposition of iron in various organs of the body and hemochromatosis.

**Total Iron Binding capacity (TIBC)** is a direct measure of the protein Transferrin which transports iron from the gut to storage sites in the bone marrow. In iron deficiency anemia, serum iron is reduced and TIBC increases.

**Transferrin Saturation** occurs in Idiopathic hemochromatosis and Transfusional hemosiderosis where no unsaturated iron binding capacity is available for iron mobilization. Similar condition is seen in congenital deficiency of Transferrin.



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| <b>LIPID PROFILE BASIC</b>                                    |         |       |                    |
| Cholesterol Total<br>(CHOD-PAP)                               | 202     | mg/dL | <200.00            |
| Triglycerides<br>(GPO-PAP)                                    | 217     | mg/dL | <150.00            |
| HDL Cholesterol<br>(Homogenous Enzymatic Colorimetric)        | 38      | mg/dL | >40.00             |
| LDL Cholesterol,Direct<br>(Homogenous Enzymatic Colorimetric) | 132     | mg/dL | <100.00            |
| VLDL Cholesterol<br>(Calculated)                              | 43      | mg/dL | <30.00             |
| Non-HDL Cholesterol<br>(Calculated)                           | 164     | mg/dL | <130.00            |

### Note

- Measurements in the same patient can show physiological & analytical variations. Three serial samples 1 week apart are recommended for Total Cholesterol, Triglycerides, HDL & LDL Cholesterol.
- Lipid Association of India (LAI) recommends screening of all adults above the age of 20 years for Atherosclerotic Cardiovascular Disease (ASCVD) risk factors especially lipid profile. This should be done earlier if there is family history of premature heart disease, dyslipidemia, obesity or other risk factors
- Triglycerides levels >150 mg/dL in fasting or >175 mg/dL in non-fasting are considered risk modifier for ASCVD risk
- Test conducted in Serum

### Treatment Goals for Lipid lowering therapy\*

| ASCVD RISK CATEGORY | TREATMENT GOAL                     |                                           |
|---------------------|------------------------------------|-------------------------------------------|
|                     | LDL-C in mg/dL<br>(Primary target) | NON HDL-C in mg/dL<br>(Co-Primary target) |
| Low                 | <100                               | <130                                      |
| Moderate            | <100                               | <130                                      |
| High                | <70                                | <100                                      |
| Very High           | <50                                | <80                                       |
| Extreme (A)         | <50<br>(<30 Optional)              | <80<br>(< 60 optional)                    |
| Extreme (B)         | <30                                | <60                                       |





Name : Mr. HIMANSHU  
Lab No. : 494249770  
Ref By : Self  
Collected : 2/3/2026 7:25:00AM  
A/c Status : P  
Collected at : VASUNDHARA LAB HOME VISIT  
GROUND FLOOR, PLOT NO.20 NEAR SBI BANK,  
VASUNDHARA, GHAZIABAD  
Contact No-01203988505

Age : 43 Years  
Gender : Male  
Reported : 2/3/2026 11:42:21AM  
Report Status : Final  
Processed at : LPL-VASUNDHARA LAB  
GF- Commercial Premises Viz 15C  
/20,Vasundhara , Ghaziabad. U.P 201012

Corporate ID : A2602260278

## Test Report

| Test Name | Results | Units | Bio. Ref. Interval |
|-----------|---------|-------|--------------------|
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### ASCVD Risk Stratification in Indian population\*

Indians are at very high risk of developing ASCVD, they usually get the disease at an early age, have a more severe form of the disease and have poorer outcome as compared to the western populations. Many individuals remain asymptomatic before they get heart attack, ASCVD risk helps to identify high risk individuals even when there is no symptom related to heart disease. Risk stratification is important to guide lipid lowering therapy and to identify treatment goals.

\*Reference: Lipid Association of India 2023 update on cardiovascular risk assessment and lipid management in Indian patients: Consensus statement IV

**Please Note:** A more formal risk assessment may be used by clinicians according to their personal preferences and familiarity with the risk scores

### Risk factors/markers

| Major ASCVD risk factors                                                                                                                                                                                                                | High-risk features                                                                                                                                                                                                                                                                                                                                                                                                                                         | Risk modifiers                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                |
|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <ol style="list-style-type: none"> <li>Age <math>\geq 45</math> years in males and <math>\geq 55</math> years in females</li> <li>Current cigarette smoking or tobacco use*</li> <li>High blood pressure*</li> <li>Low HDL-C</li> </ol> | <ol style="list-style-type: none"> <li>Family history of premature ASCVD</li> <li>CKD stage 3B or 4</li> <li>Apolipoprotein B <math>&gt; 130</math> mg/dL</li> <li>Extreme elevation of a single risk factor*</li> <li>Lipoprotein (a) <math>\geq 50</math> mg/dL</li> <li>Metabolic syndrome</li> <li>Non-alcoholic fatty liver disease with fibrosis grade 2 or 3 fibrosis</li> <li>CACS 1-99 and <math>&lt; 75^{\text{th}}</math> percentile</li> </ol> | <ol style="list-style-type: none"> <li>Lipoprotein (a) 20-49 mg/dL</li> <li>Impaired fasting glucose (fasting blood glucose 100-125 mg/dL)†</li> <li>Increased waist circumference (<math>&gt; 90</math> cm in men, <math>&gt; 80</math> cm in women)§</li> <li>hsCRP <math>&gt; 2</math> mg/L¶</li> <li>Plasma triglycerides <math>&gt; 150</math> mg/dL fasting or <math>&gt; 175</math> mg/dL non-fasting</li> <li>Rheumatoid arthritis, psoriasis, and spondyloarthropathies</li> <li>Premature menopause, pre-eclampsia, gestational diabetes, PCOS</li> <li>High polygenic risk score</li> <li>Air pollution</li> <li>Human immunodeficiency virus infection</li> </ol> |

### Risk groups

| Low risk                                                                                                                                                            | Moderate risk                                                                                                                                                                | High risk                                                                                                                                                                                                                                      | Very high risk                                                                                                                                                                                                                                                                                    | Extreme risk                                                                                                       |                                                                                                                                                                   |
|---------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| 0-1 major ASCVD risk factor, and<br>LDL-C 100-129 mg/dL, and<br>Non-HDL-C 130-159 mg/dL, and<br>Life-time CVD risk $< 30\%^*$                                       | • 2 major ASCVD risk factors, or<br>• LDL-C 130-159 mg/dL or<br>• Non-HDL-C 160-189 mg/dL or<br>• Low-risk group with $\geq 1$ risk modifier or lifetime ASCVD risk $> 30\%$ | • $\geq 3$ major ASCVD risk factors, or<br>• LDL-C 160-189 mg/dL or<br>• Non-HDL-C 190-219 mg/dL or<br>• Diabetes with 0-1 major ASCVD risk factors or<br>• 2 major ASCVD risk factor + $\geq 1$ risk modifier or<br>• Any 1 high-risk feature | • Diabetes with target organ damage<br>• Diabetes with $\geq 2$ major ASCVD risk factors<br>• CACS 100-299 or $> 75^{\text{th}}$ percentile if CACS 1-99<br>• $\geq 2$ high risk features<br>• Established ASCVD (obstructive or non-obstructive)§<br>• Heterozygous FH or LDL-C $\geq 190$ mg/dL | <b>Category A</b><br>↓<br>• ASCVD with $\geq 1$ feature of high-risk group<br>• CACS $\geq 300$<br>• Homozygous FH | <b>Category B</b><br>↓<br>• ASCVD with-<br>• $\geq 1$ feature of very high-risk group, or<br>• Recurrent ACS, or<br>• Polyvascular disease, or<br>• Homozygous FH |
| <b>Recurrent ASCVD event despite LDL-C around 30 mg/dL</b><br>These patients require special consideration, please see the text for more details- <b>Category C</b> |                                                                                                                                                                              |                                                                                                                                                                                                                                                |                                                                                                                                                                                                                                                                                                   |                                                                                                                    |                                                                                                                                                                   |



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Dr. Shweta Kasana  
MD Pathology  
Chief of Laboratory  
Dr Lal PathLabs Ltd

-----End of report -----



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#### IMPORTANT INSTRUCTIONS

•Test results released pertain to the specimen submitted. •All test results are dependent on the quality of the sample received by the Laboratory.  
•Laboratory investigations are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the Referring Physician. •Report delivery may be delayed due to unforeseen circumstances. Inconvenience is regretted. •Certain tests may require further testing at additional cost for derivation of exact value. Kindly submit request within 72 hours post reporting. •Test results may show interlaboratory variations. •The Courts/Forum at Delhi shall have exclusive jurisdiction in all disputes/claims concerning the test(s) & or results of test(s). •Test results are not valid for medico legal purposes. •This is computer generated medical diagnostic report that has been validated by Authorized Medical Practitioner/Doctor. •The report does not need physical signature.

(#) Sample drawn from outside source.

If Test results are alarming or unexpected, client is advised to contact the Customer Care immediately for possible remedial action.

Tel: +91-11-49885050, Fax: - +91-11-2788-2134, E-mail: lalpathlabs@lalpathlabs.com

